NCCP 00210 · VERSION 9B

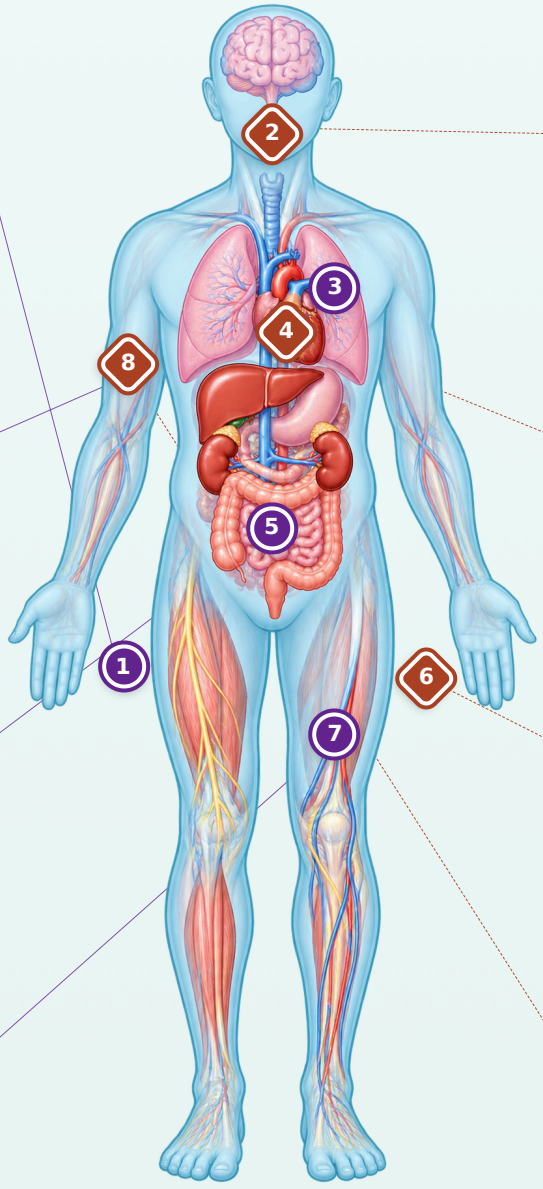
FOLFOX-4

Day 1: oxaliplatin and folinic acid, then fluorouracil bolus and a 22-hour infusion. Day 2: further fluorouracil bolus and a second 22-hour infusion; completion around Day 3. A repeating 14-day cycle. For adjuvant FOLFOX-4, NCCP describes 12 cycles; metastatic treatment continues while suitable.

Draft — independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.



1 Nerves

Oxaliplatin

Cold-triggered tingling or difficulty with buttons/walking: report.

2 Mouth / swallowing

Fluorouracil

Pain or ulcers affecting eating or drinking need advice.

3 Breathing / reaction

Treatment

New breathing difficulty requires prompt assessment.

4 Chest

Fluorouracil

New chest pain needs urgent assessment.

5 Bowel

Treatment

Diarrhoea, increased stoma output or dehydration: call early.

6 Skin / nails

Fluorouracil

Report troublesome skin changes or painful hands/feet.

7 Blood / infection

Treatment

Fever, shivers or sudden illness: contact oncology immediately.

8 Line / pump

Treatment delivery

Pain, swelling, leakage or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Fever, shivers or suddenly unwell: contact oncology immediately. Severe chest pain, breathing difficulty, collapse or seizure: 112 / 999.

NCCP 00210 · PRACTICAL SUPPORT

Your treatment, at home.

Important medicine-specific effects

Oxaliplatin

- Cold-triggered tingling / throat discomfort
- Persistent sensory neuropathy, sometimes lasting after treatment
- Allergic / infusion reaction
- Low blood counts and infection
- Nausea, diarrhoea and fatigue
- Rare serious lung or blood complications

Folinic acid

- Enhances fluorouracil activity and toxicity
- Rare allergic reaction
- Diarrhoea / mouth soreness in combination

Fluorouracil

- Diarrhoea, dehydration and mouth ulcers
- Low blood counts / infection
- Hand-foot skin soreness
- Cardiotoxicity including chest pain / ischaemia
- Severe early toxicity including DPD-related toxicity
- Pump / central-line complications

Make the plan practical

Before treatment

The team checks blood counts, kidney/liver function, symptoms, allergies and medicines. DPD testing is needed before fluorouracil unless already documented.

Bowel symptoms and fluids

Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice.

Line and pump plan

Arrange Day 2 infusion renewal and final disconnection around Day 3. This is two separate 22-hour infusions; do not use a 46-hour pump timetable.

Comfort and daily life

Avoid cold drinks and cold exposure as instructed after oxaliplatin; do not suck ice chips during fluorouracil in this regimen. Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

CUH · MON–FRI 8AM–5PM

087 181 9645

Acute Oncology Service

CUH · WEEKENDS / BANK HOLIDAYS 9AM–5PM

021 4922000

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00210/

Sources and review status are available online. Follow your own prescription and hospital instructions.